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TCGA-GM-A2DH-01A-PR

Redacted

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Malignancy		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	MB	SA
Date Reviewed	5/9/11	

REPORT - REVIEW ADDENDUM SECTION *****

DIAGNOSIS

- (A) RIGHT AXILLARY SENTINEL LYMPH NODE #1, BIOPSY:
SINGLE KERATIN-POSITIVE CELL SUSPICIOUS FOR ISOLATED TUMOR CELL IN ONE LYMPH NODE. (SEE COMMENT)
- (B) RIGHT AXILLARY SENTINEL LYMPH NODE #2, BIOPSY:
One lymph node, no tumor present (0/1).
- (C) RIGHT BREAST, TOTAL MASTECTOMY:
INVASIVE DUCTAL CARCINOMA WITH ASSOCIATED MILD LYMPHOCYTIC INFILTRATE, HIGH NUCLEAR GRADE, NOTTINGHAM HISTOLOGIC GRADE 3. (SEE COMMENT)
INVASIVE TUMOR IS LOCATED IN THE UPPER OUTER QUADRANT AND MEASURES 1.5 CM IN GREATEST DIMENSION.
FEW ADJACENT DUCTS WITH DUCTAL CARCINOMA IN SITU (DCIS), HIGH NUCLEAR GRADE, SOLID AND CRIBRIFORM TYPES WITH FOCAL COMEDONECROSIS, ASSOCIATED MICROCALCIFICATIONS, AND CANCERIZATION OF LOBULES.
Columnar cell hyperplasia with associated calcifications also adjacent to tumor.
SINGLE FOCUS OF INTRALYMPHATIC TUMOR. (SEE COMMENT)
Tumor does not approach surgical margins.
Remaining breast tissue with mild fibrocystic change.

Entire report and diagnosis completed by

COMMENT

No tumor cells are identified on the H&E stains of the sentinel lymph nodes. Immunostains for keratin were performed on each of the sentinel nodes (A1-A2, B1-B2), and a single keratin-positive cell suspicious for an isolated tumor cell is present on one of the keratin stains from sentinel node #1 (A2).

A single cluster of tumor cells is noted within a dilated lymphatic channel next to the tumor. Because the channel is quite dilated and only a single focus is identified, it is unclear whether this represents true peritumoral lymphovascular invasion or displacement artifact.

Tumor marker studies including FISH for HER2 gene copy level were reported previously (see). Because only 5% staining for estrogen receptor was reported, hormone receptor staining will be repeated on this specimen and reported in an addendum.

ICD-O-3
carcinoma, ductal, infiltrating
8500/3
Site: breast, NOS
C50.9 4012 EP

GROSS DESCRIPTION

(A) RIGHT AXILLARY SENTINEL LYMPH NODE #1, IN VIVO 195, EX VIVO 181 - A 1.5 x 1.0 x 0.6 cm possible lymph node, serially sectioned and submitted entirely for frozen section in A1, A2.

*FS/DX: NO TUMOR IDENTIFIED.

(B) RIGHT AXILLARY SENTINEL LYMPH NODE #2, IN VIVO 40, EX VIVO 28 - A 1.6 x 1.5 x 0.9 cm possible lymph node, serially sectioned and submitted entirely in B1, B2.

*FS/DX: NO TUMOR IDENTIFIED.

(C) RIGHT TOTAL MASTECTOMY - A right total mastectomy specimen (22.0 x 16.0 x 4.5 cm) with attached skin ellipse (10.0 x 5.2 x 0.5 cm). The skin ellipse contains an unremarkable nipple (1.0 cm). The specimen is sectioned from lateral to medial into ten slices with the nipple in slices 7 and 8.

An irregular (1.5 x 1.5 x 1.2 cm) solid white, gritty mass is identified in slice 6, located 2.5 cm from the deep margin, 5.2 cm from the inferior margin, 5.5 cm from the superior margin, 5.5 cm from the skin, 6.5 cm from the lateral margin, and 6.0 cm from the medial margin. The rest of the breast parenchyma is grossly unremarkable.

INK CODE: Blue - superior; orange - inferior; black - deep.

SECTION CODE: C1, superior margin; C2, inferior margin; C3, deep margin; C4, medial margin; C5, lateral margin; C6, skin closest to tumor; C7, base of nipple; C8, cross sections of tip of nipple; C9 and C10, tumor; C11, breast tissue deep to tumor;

C12, breast tissue medial to tumor; C13, breast tissue lateral to tumor; C14, breast tissue inferior to tumor; C15, breast tissue superior to tumor; C16, C17, representative sections from upper outer quadrant; C18, C19, representative sections of lower outer quadrant; C20, C21, representative sections of upper inner quadrant; C22, C23, representative sections of lower inner quadrant; C24, breast tissue anterior to tumor.

CLINICAL HISTORY

Carcinoma of right breast.

SNOMED CODES

T-04050, M-85002, M-85003, T-C4714, M-85017

"Some tests reported here may have been developed and performance characteristics determined by
These tests have not been specifically cleared or approved by the U.S. Food and Drug Administration."

Released by:

Start of ADDENDUM

ADDENDUM

Addendum completed by

This report is issued to give immunohistochemistry results.

Immunohistochemical staining is performed on a representative formalin-fixed, paraffin-embedded section of INVASIVE DUCTAL CARCINOMA, right breast, block C9.

MARKER	RESULTS	% POSITIVE
Estrogen Receptor	Low Positive (1-9%)	5 %
Progesterone Receptor	Negative	0 %
Ki-67	High (>35%)	approx. 50 %

COMMENT

See : for HER2 immunostaining and FISH results.

FOOTNOTE

Estrogen receptor was assessed by immunohistochemistry using antibody 6F11

Progesterone receptor was assessed by immunohistochemistry using antibody PgR1294.

Ki-67 was assessed by immunohistochemistry using antibody MIB1

Released by:

-----END OF REPORT-----